

decisions” and are thus, not immune from liability under section 820.2. (*Id.* at 1058.) Courts have since distinguished or limited *Elton*’s holding. In *Ronald S. v. County of San Diego* (1993) 16 Cal.App.4th 887, the court explained that *Elton* can be distinguished by considering that “[t]he maintenance of a child in a foster home involves an obligation of continued supervision by the County. The child remains a ward of the County. Much of what the County is obligated to do in terms of continued administration of the child’s welfare undoubtedly constitutes simple and uncomplicated surveillance which reasonably could be characterized as ministerial.” (*Id.* at 898.) Further, “*Elton* was decided in 1970, many years before the adoption of current statutes (such as Welf. & Inst. Code, § 16501.1) requiring that social workers engage in a discretionary analysis of the needs and interests of a dependent child.” (*Becerra v. County of Santa Cruz* (1998) 68 Cal.App.4th 1450, 1464.) Thus, while *Elton* appears to make a broad statement regarding the nondiscretionary actions, the Court finds *Elton* does not apply here because the laws requiring a discretionary analysis of social workers have changed, and here, the allegations are not based upon the social workers failing to perform uncomplicated surveillance.

Here, the Court finds that *K.C.*’s analysis of the discretionary immunity test under *Caldwell* is more persuasive than *D.G.*’s analysis. Therefore, the Court finds that where *K.C.* and *D.G.* conflict, the Court will follow *K.C.* Here, the allegations are nearly identical to the ones in *K.C.* Plaintiffs allege that they informed their social workers that they were experiencing abuse by their foster father, but nothing was done. A fair reading of these allegations includes that the County’s social workers were confronted with reports of sexual abuse that should have prompted investigative or corrective action, but they failed to properly exercise their discretion to do so. Under *K.C.* these allegations make the claims against the County immune under discretionary immunity and therefore, the Court finds that the negligence claim against the County is **barred**.

The County also argues that the complaint is uncertain. However, uncertainty is not a ground available in a motion for judgment on the pleadings. Code of Civil Procedure section 438(c)(2) states that “The motion provided for in this section may be made as to either of the following: (A) The entire complaint or cross-complaint or as to any of the causes of action stated therein. (B) The entire answer or one or more of the affirmative defenses set forth in the answer.” (Code of Civil Procedure section 438(c)(2).) The “uncertainty” of a pleading may be raised in a special demurrer, but “furnishes no ground for support of a judgment on the pleadings.” (*Bates v. Escondido Union High School Dist.* (1933) 133 Cal.App.725, 731.) A party’s failure to raise the defect of uncertainty by demurrer constitutes a waiver of that objection. (*Stockton Newspapers, Inc. v. Redevelopment Agency* (1985) 171 Cal.App.3d 95, 103.)

13. 9:00 AM CASE NUMBER: MSC22-00100
CASE NAME: RIDEAU VS GHC OF CONTRA COSTA, ET AL.
HEARING ON SUMMARY MOTION
FILED BY: GHC OF CONTRA COSTA, LLC
TENTATIVE RULING:

The Motion of Defendants GHC of Contra Costa, LLC, Life Generations Healthcare, and Ann Nguyen-Traxler, M.D. for Summary Judgment or, in the Alternative, Summary Adjudication of Issues is **denied**

for the reasons that follow.

I. Background

This action was filed on January 14, 2022. Plaintiff Marlene Rideau, individually and as successor in interest to the estate of her son Dwayne Rideau ("Decedent" or "Mr. Rideau"), brings this action against defendants GHC of Contra Costa, LLC, Life Generations Healthcare, and Dr. Ann Nguyen-Traxler, M.D., arising from care and treatment provided to Mr. Rideau at Bayberry Skilled Nursing and Healthcare Center ("Bayberry"). Plaintiff contends that her son, a wheelchair-bound resident of Bayberry, died of liver cancer that would have been detected and treated in time to save his life had defendants maintained the cancer surveillance protocol established by his treating hepatologist. GHC of Contra Costa, LLC and Life Generations Healthcare are the owners and operators of Bayberry (collectively, the "Facility Defendants"). Dr. Nguyen-Traxler served as Mr. Rideau's attending physician and as Medical Director of Bayberry during the period at issue. Defendants now move for summary judgment, or in the alternative summary adjudication of issues, contending that no act or omission by Dr. Nguyen-Traxler or Bayberry's nursing staff fell below the applicable standard of care, that Decedent's death resulted from the natural progression of his underlying liver disease rather than any failure to diagnose or treat cancer, and that no conduct by any defendant rose to the level of recklessness required to sustain the elder abuse cause of action.

The operative Second Amended Complaint alleges four causes of action. At all relevant times Mr. Rideau was a "dependent adult" within the meaning of the Elder Abuse and Dependent Adult Civil Protection Act ("EADCPA"), Welfare and Institutions Code section 15600, et seq. The first cause of action for wrongful death due to medical negligence and the fourth cause of action for neglect, brought as a survival claim for professional medical negligence on behalf of Decedent, are asserted against all defendants. The second cause of action for reckless or willful neglect of a dependent adult, or neglect with fraud, under the EADCPA is asserted against all defendants. The third cause of action for violation of the Resident Bill of Rights is asserted against the Facility Defendants only.

Decedent Dwayne Rideau was diagnosed at age 46 with end-stage liver disease secondary to hepatitis C and alcohol use, cirrhosis of the liver, hepatic encephalopathy, an upper gastrointestinal bleed due to esophageal variceal bleeding, portal hypertension, and thrombocytopenia during an admission to California Pacific Medical Center ("CPMC"). During that admission Decedent was evaluated for liver transplantation but was denied. (Defs.' Separate Statement of Undisputed Material Facts ["UMF"] Nos. 5-6.)

On January 14, 2014, Decedent was admitted to Bayberry following a medical event resulting in left-sided paralysis. The admitting physician, Hamta Jafari, M.D., documented Decedent's chronic conditions including hepatitis C, alcoholic cirrhosis, thrombocytopenia secondary to cirrhosis, left-sided hemiparesis, and hypertension. Decedent weighed approximately 200 pounds at admission. Dr. Jafari served as Decedent's attending physician at Bayberry until January 13, 2016, when Dr. Ann Nguyen-Traxler assumed that role. (UMF No. 10.)

On October 30, 2014, Decedent established specialty care with Chanda Ho, M.D., a hepatologist and gastroenterologist at the Sutter Health liver disease clinic, for evaluation and management of his liver disease. Decedent's mother Marlene Rideau and his sister were present. Dr. Ho reported that Decedent's cirrhosis was complicated by hepatic encephalopathy, portal hypertension, and variceal

bleeding post-banding, and that his extremely limited mobility from paralysis ruled out liver transplantation. At this visit Dr. Ho initiated a liver cancer surveillance protocol given Decedent's cirrhosis, consisting of abdominal ultrasounds and alpha fetoprotein ("AFP") level testing every six months. Dr. Ho completed order forms for these tests as well as routine lab work and provided the forms to Decedent to take back to Bayberry. Bayberry executed Dr. Ho's orders, obtained the abdominal ultrasound and lab work, and forwarded the results to Dr. Ho, as documented in Dr. Ho's progress note for the follow-up visit on December 19, 2014. (UMF No. 11.)

On June 10, 2015, Decedent returned to Dr. Ho's liver clinic for a six-month follow-up visit. Dr. Ho's progress note, which was mistakenly addressed and faxed to Dr. Jafari, documented her plan for lab work every three months and AFP testing at least every six months for HCC screening, and requested the results be faxed to her. Dr. Ho asked that the next abdominal ultrasound be performed at CPMC and requested Decedent return in six months. The parties dispute whether Dr. Nguyen-Traxler received a copy of this note. Defendants contend the note was faxed to Dr. Jafari's outdated fax number and was not received by Dr. Nguyen-Traxler. Plaintiffs contend a copy of the note is present in Decedent's Bayberry medical chart. (UMF No. 13 and Plf.'s Response.)

On or about January 13, 2016, Dr. Nguyen-Traxler assumed care of Decedent as his attending primary care physician at Bayberry. In her first progress note of that date, Dr. Nguyen-Traxler documented that Decedent was being followed by a gastroenterologist for his hepatitis C and alcoholic cirrhosis, that he was receiving liver cancer surveillance including AFP testing, and that there were no signs of hepatocellular carcinoma. (UMF No. 16.)

On February 3, 2016, Decedent had his fourth visit with Dr. Ho at the liver disease clinic. Dr. Ho's progress note (again addressed to Dr. Jafari) documented that the July 2015 abdominal ultrasound showed cirrhosis with evidence of portal hypertension, reversible flow in the main portal vein, splenomegaly, and a partially occluded thrombus in the main portal vein, with no focal liver lesions identified. Dr. Ho reiterated her plan for AFP testing and abdominal ultrasounds every six months for liver cancer screening and provided Decedent with order forms. The parties dispute whether Dr. Nguyen-Traxler had any role in ordering liver cancer screening tests. Defendants contend Dr. Ho's orders were carried out by Bayberry staff under the specialist's direction. Plaintiffs contend that under Dr. Ho's treatment recommendations the primary care physician was responsible for ordering the AFP and abdominal ultrasound tests on the recommended schedule and communicating results back to Dr. Ho and that only Dr. Nguyen-Traxler, whose medical license was associated with Bayberry, could place those orders. (UMF No. 17 and Plf.'s Response.)

On May 20, 2016, Decedent underwent an abdominal ultrasound at CPMC for HCC screening. No hepatic mass was identified. A 10 mm cyst was present in Segment 6 and a 1.6 cm simple cyst in Segment 3. The portal vein measured 10 mm with a partially occluded thrombus unchanged from 2015. (UMF No. 18.)

On July 14, 2016, Decedent was admitted to CPMC for an EKG, lab work, and tissue biopsy on Dr. Ho's orders, following which Dr. Ho referred Decedent to CPMC for treatment of large gastric varices. (UMF No. 19.) On September 23, 2016, Decedent was admitted to CPMC for an endoscopic ultrasound revealing multiple varices, which were treated with glue and coiling. Endosonographic imaging of the left lobe of the liver showed cirrhosis but no mass. (UMF No. 20.)

On November 4, 2016, Decedent had a clinic visit with Dr. Ho. Dr. Ho reiterated her plan for AFP testing every six months and noted Decedent was due for an abdominal ultrasound in November 2016. The parties dispute who ordered the AFP testing performed around this time. Defendants contend the orders were Dr. Ho's. Plaintiffs contend the AFP testing reports reflect that Dr. Nguyen-Traxler (not Dr. Ho) ordered the tests, consistent with Dr. Nguyen-Traxler's testimony that only she could place orders at Bayberry. (UMF No. 21 and Plf.'s Response.) Following this visit, Bayberry staff documented that Marlene Rideau said she would arrange her son's ultrasound appointment and next appointment with Dr. Ho, and would provide the ultrasound results to Bayberry to fax to Dr. Ho. (UMF No. 22.)

AFP lab work was obtained on November 9, 2016, with a normal result of 5.2. An abdominal ultrasound was performed at Bayberry on November 11, 2016, and results were faxed to the liver clinic on November 15, 2016. Dr. Nguyen-Traxler's November 10, 2016 progress note states "labs drawn by GI physician, reviewed." Plaintiffs dispute that Dr. Ho ordered those labs, noting the AFP report identifies Dr. Nguyen-Traxler as the ordering physician. (UMF No. 23 and Plf.'s Response.)

Subsequent visits with Dr. Ho occurred on May 17, 2017, and September 20, 2017. At each visit Dr. Ho provided order forms for lab work and abdominal ultrasounds and requested follow-up in five to six months. The parties continue to dispute who actually placed the orders for AFP testing and abdominal ultrasounds at Bayberry, with Plaintiffs consistently contending that Dr. Nguyen-Traxler was the ordering physician because only she could place orders at Bayberry. (UMF Nos. 25-29 and Plf.'s Responses.) This dispute is material to the question of whether Dr. Nguyen-Traxler understood herself to have an independent obligation to maintain the HCC surveillance schedule.

On April 12, 2018, Decedent returned to the liver clinic and was seen by Dr. Ho's nurse practitioner, Tammy K. Lee, N.P. NP Lee's management plan included AFP level testing, an abdominal ultrasound, an endoscopic ultrasound at CPMC, and lab work for MELD monitoring, all to be completed before Decedent's return to the clinic in six months. NP Lee prepared a one-time lab order form for AFP and a one-time imaging requisition for an abdominal ultrasound, with the plan that Bayberry would execute the orders and fax results back to the liver clinic. NP Lee's progress note was faxed to Dr. Jafari at the outdated fax number. The parties dispute whether Bayberry could execute these orders without Dr. Nguyen-Traxler's involvement. Plaintiffs contend and Dr. Nguyen-Traxler testified that she would place the orders for the screening tests, the results would come back to her, and Bayberry staff would fax the results to Dr. Ho's office. (UMF Nos. 33-37 and Plf.'s Responses.)

AFP lab work was obtained on April 20, 2018 and June 19, 2018. The June 2018 results were faxed to the liver clinic. The June 2018 AFP was the last AFP test performed on Decedent prior to November 2020. On September 8, 2018, Decedent underwent an abdominal ultrasound at Bayberry. The results were faxed to the liver clinic on November 14, 2018. The September 2018 ultrasound was the last abdominal ultrasound performed on Decedent as part of his HCC surveillance protocol prior to his emergency department visit in September 2019. (UMF Nos. 39-40.)

Decedent's return appointment with Dr. Ho was scheduled for November 21, 2018. On November 14, 2018, the liver clinic confirmed the appointment with Bayberry, and Bayberry confirmed that transportation had been arranged. Bayberry faxed Decedent's most recent lab results and ultrasound report to the liver clinic. On November 14, 2018, Bayberry staff advised Marlene Rideau of the

upcoming appointment. However, on November 15, 2018, Ms. Rideau called Bayberry and asked to cancel the appointment. The appointment was cancelled. No follow-up appointment with Dr. Ho was scheduled. As a result, Decedent was “lost” to follow-up with the liver clinic. He never returned to the clinic, and Dr. Ho made no further orders for liver disease management or HCC screening. (UMF Nos. 41, 43-45.)

The parties dispute whether Bayberry had an independent obligation to follow up and reschedule the cancelled November 2018 appointment. Defendants contend it was reasonable to rely on Marlene Rideau’s statement that she would reschedule and that no one informed Dr. Nguyen-Traxler of the lapse in specialist follow-up. Plaintiffs contend that Bayberry (not Decedent’s 81-year-old mother, Marlene) bore responsibility to coordinate specialist appointments and arrange paratransit for a wheelchair-bound dependent adult resident, that it was foreseeable an elderly family member might fail to reschedule, and that Bayberry’s failure to notice and act when nearly two years passed without a specialist appointment fell below the standard of care.

Following the November 2018 cancellation, no AFP testing and no abdominal ultrasound for HCC surveillance were performed on Decedent for approximately two years. However, during this period, Dr. Nguyen-Traxler continued to make monthly progress notes stating, “AFP normal, no evidence of HCC or hepatoma.” Plaintiffs contend that the notations were false and Dr. Nguyen-Traxler had no factual basis for those representations.

On September 17, 2019, Dr. Nguyen-Traxler sent Decedent to the John Muir Medical Center Emergency Department for evaluation of flank pain. On September 18, 2019, an abdominal ultrasound was performed in the ED. The radiologist reported a potential lack of flow in the portal vein and identified a finding in the right lobe of the liver, recommending an abdominal CT to investigate portal vein flow. The CT scan obtained on September 18, 2019 was read as negative with no recommended follow-up. Decedent was diagnosed with a urinary tract infection and discharged on antibiotics. (UMF No. 47.)

The parties dispute the significance of the September 2019 imaging, and that dispute is central to the causation issues in this case. The radiologist who performed the September 2019 ultrasound reported the finding in the right lobe as cysts. Plaintiff’s medical expert Dr. Arora opines the finding was a 2.0 cm lesion inconsistent with a simple cyst and was highly suspicious for early stage HCC. Defense expert Dr. Gaensler opines the finding was a benign cyst meeting simple cyst criteria and required no follow-up. (UMF No. 47 and Plf.’s Response.)

The parties further dispute what the September 18, 2019 CT scan shows. When defense experts Dr. Hendifar and Dr. Gaensler reviewed the September 2019 CT retrospectively during litigation, Dr. Gaensler identified a 5.91 x 4.10 cm tumor in the hilum of the liver extending into the left lobe with portal vein tumor thrombus and bile duct dilatation, and opined the cancer was already untreatable as of September 2019. Plaintiff’s expert Dr. Arora disputes this finding, opining that the area identified by Dr. Gaensler measured substantially smaller on subsequent 2020 and 2021 imaging studies, shrinking from the claimed 5.91 x 4.10 cm to approximately 3.6 x 3.2 cm on the November 2020 CT and 3.4 x 3.2 cm on the January 2021 CT, which Dr. Arora states is inconsistent with an untreated malignant tumor. (UMF No. 48 and Plf.’s Response.)

In April 2020, Decedent weighed 272 pounds against an ideal body weight of 178 pounds and was

placed on a weight loss diet. When Dr. Nguyen-Traxler saw Decedent for her monthly visit on August 20, 2020, she noted a significant weight loss of 42 pounds in the previous six months. Decedent reported he did not like the food at Bayberry, often skipped breakfast, and denied abdominal discomfort. Dr. Nguyen-Traxler attributed the weight loss to desired weight reduction, dietary changes, reduced outside food due to pandemic restrictions, and depression from isolation. (UMF Nos. 49-52.)

The parties dispute whether Bayberry's nursing staff responded appropriately to Decedent's weight loss during the June through September 2020 period. Defendants contend Bayberry records show Decedent was alert and responsive with no complaints of pain or discomfort, and that his food intake varied from day to day. Plaintiffs contend that a Bayberry nurse documented that Decedent had been eating poorly since June 2020, that Bayberry continued to characterize his weight loss as desirable through October 2020, and that Bayberry failed to inform Decedent's family of his significant weight loss during the June through September 2020 period, preventing the family from advocating on his behalf. (UMF No. 53 and Plf.'s Response.)

On or about October 23, 2020, Dr. Nguyen-Traxler was told by someone at Bayberry, possibly the dietician, that Decedent's weight loss had occurred over three months rather than six, which caused her to suspect malignancy. Dr. Nguyen-Traxler ordered weekly weights, blood work, and an abdominal ultrasound. Decedent's mother, Marlene Rideau, was informed and agreed to the testing. (UMF No. 54.)

An abdominal ultrasound performed on October 28, 2020 was interpreted as suspicious for an ill-defined 3 cm mass in the right liver lobe. Dr. Nguyen-Traxler ordered an abdominal and pelvic CT with contrast. (UMF No. 55.) The CT was performed by NorCal Imaging on November 6, 2020. In a report prepared on November 9, 2020, radiologist Tereasa Simonson, M.D. found no evidence of an enhancing liver mass. Relying on Dr. Simonson's negative read, Dr. Nguyen-Traxler spent the following eight weeks seeking alternative explanations for Decedent's continued weight loss and ordered an urgent referral to a gastroenterologist. (UMF No. 58.)

Over the following eight weeks Decedent was evaluated at the County clinic by two primary care providers before being seen by gastroenterologist Dr. Zheng on December 3, 2020. AFP tumor marker testing ordered on November 30, 2020 resulted on December 1, 2020 with an abnormally elevated level consistent with liver cancer, the first abnormal AFP in Decedent's testing history, all prior results having been normal. (UMF Nos. 61-62.)

In hindsight, the November 6, 2020 CT study showed untreatable metastatic hepatocellular cancer. On January 4, 2021, when Dr. Aaron Hayashi, a radiologist at Contra Costa Regional Medical Center, retrospectively reviewed the November 6, 2020 CT, he identified a 3.2 x 3.2 cm malignant mass in the posterior right lobe, a 10.3 x 11.3 cm malignant mass in the right lobe, a malignant tumor in the main portal vein, and pulmonary nodules representing metastatic liver cancer. These findings had not been identified by Dr. Simonson in her original November 9, 2020 interpretation. (UMF No. 57.)

On December 16, 2020, Dr. Ho conducted a virtual visit with Decedent and Marlene Rideau and recommended resuming AFP labs and abdominal ultrasounds every six months going forward. A follow-up appointment with Dr. Ho was scheduled for June 16, 2021. (UMF Nos. 64-65.)

The parties dispute Decedent's condition during the final months of his life. Defendants contend

Bayberry records from June 1, 2020 through January 4, 2021 reflect normal vital signs and no pain complaints related to liver disease. Plaintiffs contend the records from the last month of Decedent's life reflect pain from advanced liver cancer. (UMF No. 66 and Plf.'s Response.)

On January 4, 2021, Decedent was difficult to arouse. Dr. Nguyen-Traxler and Marlene Rideau were contacted and agreed to transfer Decedent to the emergency department at Contra Costa Regional Medical Center. Bayberry provided the treating physicians with recent history including the November 2020 CT study, which had been read as negative for liver masses. (UMF No. 67.)

At Contra Costa Regional Medical Center, Dr. Hayashi reviewed both the new January 4, 2021 non-contrast CT and the prior November 6, 2020 contrast CT. In his retrospective review of the November 2020 study Dr. Hayashi identified liver masses suspicious for HCC and pulmonary nodules consistent with metastasis. Decedent was admitted for treatment of sepsis. (UMF No. 69.)

The clinical assessment was incurable metastatic liver cancer consistent with HCC and hepatic biliary obstruction. As Decedent had advanced incurable disease and was a poor candidate for treatment, home hospice was recommended. The family agreed, and Decedent was discharged on January 7, 2021. Decedent died on January 14, 2021. (UMF No. 70.)

II. Summary Judgment Standard

"[F]rom commencement to conclusion, the party moving for summary judgment bears the burden of persuasion that there is no triable issue of material fact and that he is entitled to judgment as a matter of law." (*Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 850.) As to each claim as framed by the complaint, a defendant moving for summary judgment or summary adjudication by doing one of the following: (1) presenting affirmative evidence negating an essential element of plaintiff's cause of action; (2) showing a complete defense; or (3) showing that plaintiff does not possess and cannot reasonably obtain needed evidence. (Code Civ. Proc. §437c(p)(2).)

Once the defendant has met that burden, the burden shifts to the plaintiff to show that a triable issue of one or more material facts exists as to that cause of action or a defense thereto. To establish a triable issue of material fact, the party opposing the motion must produce substantial responsive evidence. (*Sangster v. Paetkau* (1998) 68 Cal.App.4th 151, 166.) Nevertheless, courts "liberally construe the evidence in support of the party opposing summary judgment and resolves doubts concerning the evidence in favor of that party." (*Dore v. Arnold Worldwide, Inc.* (2006) 39 Cal.4th 384, 389.)

"A motion for summary adjudication may be made by itself or as an alternative to a motion for summary judgment and shall proceed in all procedural respects as a motion for summary judgment." (Code Civ. Proc. § 437c, subd. (f)(2).)

III. Evidentiary Matters

A. Judicial Notice

On September 9, 2025, Defendants filed a request for judicial notice of documents filed in support of Defendant Nguyen-Traxler's motion for summary judgment or, in the alternative, summary adjudication filed on May 7, 2025, including the declarations of Ann Nguyen-Traxler, M.D., Andrew Hendifar, M.D., Erik H.L. Gaensler, M.D., and Sonja Rosen, M.D., together with the exhibits attached

to those declarations.

On January 9, 2026, Plaintiffs filed a request for judicial notice of documents filed in opposition to Defendant Nguyen-Traxler's motion for summary judgment or, in the alternative, summary adjudication on September 25, 2025, including the declarations of Felicia C. Curran, Steven H. Fugaro, M.D., Gregory M. Graves, M.D., and Lokesh C. Arora, M.D., together with the exhibits attached to those declarations.

Both requests are granted, although a formal request for judicial notice of materials previously filed in this action is not required. It is sufficient to call the Court's attention to such papers. (Weil & Brown, Cal. Practice Guide: Civil Procedure Before Trial (The Rutter Group 2023) ¶ 9:53.1a.)

B. Plaintiff's Objections to Evidence

Plaintiff's objection to the Supplemental Declaration of Andrew Hendifar, M.D. is **sustained**. The supplemental Hendifar declaration attacks the qualifications of Dr. Fugaro, Dr. Graves, and Dr. Arora, arguing that none of them has the specialized expertise in liver disease management necessary to opine on the issues presented in this case. Paragraphs 8.1 through 8.5 of the declaration expand Defendants' causation argument by introducing new detail in support of the argument that Decedent was never definitively diagnosed with HCC so Plaintiffs cannot prove causation. New matter is not permitted with reply papers. (Code Civ. Proc. § 437c(b)(4); *Carbajal v. CWPSC, Inc.* (2016) 245 Cal.App.4th 227, 241.)

Plaintiff's Objection Nos. 1-3 to the Declaration of Sonja Rosen, M.D., are **sustained**. Dr. Rosen's opinion that Dr. Nguyen-Traxler met the standard of care applicable to a medical director of a skilled nursing facility is conclusory and lacking in foundation. (*McAlpine v. Norman* (2020) 51 Cal.App.5th 933, 939-941.)

C. Defendants' Objections to Evidence

On October 6, 2025, Defendant Nguyen-Traxler, M.D. filed "Evidentiary Objections and Motion to Strike the Declarations of Steven H. Fugaro, M.D., Gregory M. Graves, M.D., and Lokesh K. Arora, M.D." On January 16, 2026, the Facility Defendants filed a notice of joinder in those objections and separately moved to strike the Supplemental Declaration of Dr. Fugaro filed on January 9, 2026.

Declaration of Steven H. Fugaro, M.D. Overruled. Dr. Fugaro is qualified to offer standard-of-care opinions, and the issues raised by defendants go to weight rather than admissibility. Any factual conflicts are for cross-examination and do not provide a basis for exclusion.

Declaration of Gregory M. Graves, M.D. Overruled. Dr. Graves' training and experience adequately support his opinions, and any gaps concerning comorbidities affect weight, not admissibility.

Declaration of Lokesh C. Arora, M.D. Overruled. Dr. Arora's experience in liver-cancer diagnosis and treatment provides sufficient foundation for his opinions. Concerns regarding comorbidities or diagnostic assumptions go to weight, not admissibility.

Supplemental Declaration of Steven H. Fugaro, M.D. Overruled. The original and supplemental declarations together provide adequate foundation for the opinions expressed.

IV. Analysis

A. First and Fourth Causes of Action for Negligence (Issue Nos. One and Four)

Defendants seek summary adjudication of the First and Fourth Causes of Action for Negligence and Wrongful Death on grounds that Plaintiffs cannot prove a breach of the standard of care or causation. Because both causes of action turn on the same issues, the Court addresses them together.

1. Legal Standard for Medical Malpractice Actions

The elements to a claim for medical negligence are (1) a duty to use such skill, prudence, and diligence as other members of the profession commonly possess and exercise, (2) a breach of the duty, (3) a proximate causal connection between the negligent conduct and the injury, and (4) resulting loss or damage." (*Johnson v. Superior Court* (2006) 143 Cal.App.4th 297, 305.)

Expert testimony is generally required in every professional negligence case to establish the applicable standard of care, whether the defendant met or breached that standard, and whether the defendant's negligence caused the plaintiff's damages. (*Sanchez v Kern Emergency Med. Transp. Corp.* (2017) 8 Cal.App.5th 146, 153.)

"Whenever the plaintiff claims negligence in the medical context, the plaintiff must present evidence from an expert that the defendant breached his or her duty to the plaintiff and that the breach caused the injury to the plaintiff." (*Powell v. Kleinman* (2007) 151 Cal.App.4th 112, 123.) "California courts have incorporated the expert evidence requirement into their standard for summary judgment in medical malpractice cases. When a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence." (*Munro v. Regents of University of California* (1989) 215 Cal.App.3d 977, 984-985.)

A judge should strictly construe the declarations in support of the motion and should liberally construe the opposing declarations. (*Jennifer C. v Los Angeles Unified Sch. Dist.* (2008) 168 Cal.App.4th 1320, 1332 [the requirement of detailed, reasoned explanation for expert opinions applies to expert declarations submitted in support of summary judgment, not to those submitted in opposition].)

2. Defendants' Evidence

Defendants rely primarily on the declarations of Sonja Rosen, M.D., an internist and geriatrician who opines on the standard of care applicable to Dr. Nguyen-Traxler and Bayberry's nursing staff, Andrew Hendifar, M.D., an oncologist who opines on standard of care and causation, and Erik H.L. Gaensler, M.D., a diagnostic and interventional radiologist who opines on the imaging findings. Together, these declarants opine that Dr. Nguyen-Traxler complied with the standard of care because she was not required to independently manage HCC surveillance, that she reasonably relied on Decedent's outside specialist for disease management, and that Decedent's death resulted from end-stage liver disease rather than any delay in diagnosis or treatment of cancer.

Dr. Rosen opines that Dr. Nguyen-Traxler met the standard of care in her roles as both attending physician and medical director. (Rosen Decl., ¶ 7.) With respect to HCC surveillance, Dr. Rosen opines that the standard of care did not require Dr. Nguyen-Traxler to independently order AFP testing or abdominal ultrasounds because Decedent's liver disease, including cancer surveillance, was being managed by Dr. Ho and her liver clinic. (*Id.*, ¶¶ 5(c), 5(e), 9.) Dr. Rosen further opines that it was

reasonable for Dr. Nguyen-Traxler to assume Dr. Ho was continuing to manage Decedent's liver care after the November 2018 appointment cancellation, as no one informed her of the lapse. (*Id.*, ¶¶ 5(i), 9.) Dr. Rosen additionally opines that standing orders for periodic screening tests are not permitted in skilled nursing facilities, and Dr. Ho's one-time orders for AFP testing and abdominal ultrasound issued at the April 2018 visit did not create an ongoing independent obligation for Dr. Nguyen-Traxler to continue ordering the testing. (*Id.*, ¶ 9.)

With respect to the weight loss, Dr. Rosen opines that it was reasonable for Dr. Nguyen-Traxler to attribute the August 2020 weight loss to Decedent's obesity, dietary changes, and pandemic-related isolation, particularly given that Decedent was asymptomatic and she reasonably believed Dr. Ho was continuing to monitor him for liver cancer. (*Id.*, ¶¶ 5(j), 10.) Dr. Rosen opines that Dr. Nguyen-Traxler's response upon learning of accelerated weight loss in October 2020 was prompt and appropriate, and that the subsequent two-month delay in diagnosis resulted from the radiologist's misinterpretation of the November 2020 CT study rather than any failure by Dr. Nguyen-Traxler. (*Id.*, ¶¶ 5(k), 5(l), 10.)

With respect to the medical director role, Dr. Rosen states that the standard of care applicable to a medical director did not require Dr. Nguyen-Traxler to identify the cause of Decedent's weight loss or to oversee individual attending physician treatment decisions. (*Id.*, ¶¶ 11-12.)

Dr. Gaensler opines that “while it is possible that Mr. Rideau's cancer was cholangiocarcinoma, it is more likely than not that Mr. Rideau had HCC given the history of cirrhosis and the fact that cholangiocarcinoma is rare.” (Gaensler Decl., ¶ 6.) Dr. Gaensler further states that his retrospective review of the September 2019 CT study reveals a 5.91 x 4.10 cm malignant tumor in the hilum of the liver that “that is likely hepatocellular carcinoma.” (*Id.*, ¶¶ 8.5, 8.5(c), 8.5(f) 8.5(g).) Dr. Gaensler opines that as of September 2019 this tumor was not amenable to any treatment given its size, location, and the severity of the underlying cirrhosis and varices. (*Id.*, ¶ 8.6.) Dr. Gaensler also opines that the 2.0 cm hypoechoic lesion identified on the September 2019 abdominal ultrasound was a benign cyst based on its anechoic appearance, round shape, well-defined margins, did not require further investigation or follow-up. (*Id.*, ¶ 8.3.)

Dr. Hendifar opines that Dr. Nguyen-Traxler met the standard of care for an internal medicine physician throughout her treatment of Mr. Rideau from January 2016 through January 2021. (Hendifar Decl., ¶ 10.0.) He states that the management of Mr. Rideau's liver disease and HCC surveillance was solely the responsibility of clinicians at the Sutter Health liver clinic, who never delegated that responsibility to Dr. Nguyen-Traxler. Dr. Hendifar states that Dr. Nguyen-Traxler was unaware of Dr. Ho's management and treatment plan because Dr. Ho's progress notes were faxed Dr. Jafari rather than Dr. Nguyen-Traxler (*Id.*, ¶¶ 10.1, 10.2).

Dr. Hendifar opines that the standard of care did not require Dr. Nguyen-Traxler to recognize that Mr. Rideau had stopped attending the liver clinic, or to independently order HCC screening, because Dr. Ho and Nurse Practitioner Lee never provided standing orders for HCC surveillance, NP Lee testified that her April 2018 orders for AFP testing and abdominal ultrasound were one-time orders, and standing orders for special labs and imaging are generally not allowed at skilled nursing facilities (*Id.*, ¶¶ 10.3, 10.4.) Dr. Hendifar states that Dr. Nguyen-Traxler responded reasonably and within the standard of care to Mr. Rideau's weight loss beginning in late August 2020, and reasonably attributed

the initial weight loss to diet changes and pandemic-related isolation until October 2020, when she suspected malignancy and began a diagnostic workup (*Id.*, ¶ 10.5.) Dr. Hendifar states Dr. Nguyen-Traxler 's decision making from October 2020 through January 2021, including her reliance on a negative CT interpretation, met the standard of care. (*Id.*, ¶ 10.6.)

Dr. Hendifar states that it was reasonable for physicians to presume that Mr. Rideau's cancer was HCC given its strong association with chronic liver disease and cirrhosis. Nevertheless, he opines that plaintiff cannot prove HCC definitively, because no biopsy was performed, no autopsy was conducted, and the imaging is not definitive. (*Id.*, ¶ 8.)

Dr. Hendifar further opines that no act or omission by Dr. Nguyen-Traxler or Bayberry was a substantial factor in causing Mr. Rideau's death. (*Id.*, ¶ 12.0.) Based on his review of the imaging studies and his collaboration with Dr. Gaensler, Dr. Hendifar states that as of September 2019 Decedent already had a 5+ cm tumor in the liver hilum involving the portal vein and biliary tree that was not amenable to any treatment given its size, location, and his severe underlying comorbidities. (*Id.*, ¶¶ 12.1, 12.5.) Even had the cancer been diagnosed earlier, there was no curative treatment available that would have given Decedent a better than 50% chance of survival, and an earlier diagnosis would not have improved his quality of life or altered the timing of his death. (*Id.*, ¶¶ 12.4, 12.5.) Dr. Hendifar states that it is speculative whether earlier HCC screening would even have resulted in an earlier diagnosis, given that both CT studies performed in 2019 and 2020 were interpreted as negative for liver lesions, and AFP testing is an unreliable screening tool with limited sensitivity. (*Id.*, ¶ 12.7.)

Defendants' expert declarations make a prima facie showing that the standard of care did not require Dr. Nguyen-Traxler to independently manage HCC surveillance, that she reasonably relied on Decedent's outside specialist for disease management, and that Decedent's death resulted from advance and incurable liver disease and not a delay in diagnosis or treatment. The Court therefore finds that Defendants met their initial burden under Code of Civil Procedure section 437c(p)(2), and the burden shifts to Plaintiffs to produce evidence raising a triable issue of material fact.

3. Plaintiff's Evidence

Plaintiff's opposition is supported by declarations of Steven H. Fugaro, M.D., an internist who opines on the standard of care applicable to Dr. Nguyen-Traxler as a primary care physician managing a patient with an outside specialist, Gregory M. Graves, M.D., an oncological surgeon who opines on causation, available treatment options, and prognosis, and Lokesh C. Arora, M.D., a diagnostic and interventional radiologist who opines on the imaging findings and treatability. Together, these declarants opine that Dr. Nguyen-Traxler breached the standard of care by failing to maintain the HCC surveillance protocol established by Dr. Ho, that Bayberry's nursing staff independently breached the standard of care by failing to reschedule the cancelled November 2018 specialist appointment and by failing to inform Decedent's family of his significant weight loss, and that had surveillance been maintained, Decedent's liver cancer would have been detected when it was still amenable to treatment with a better than 50 percent chance of survival.

Dr. Fugaro opines that Dr. Nguyen-Traxler fell below the standard of care in her care and treatment of Decedent from June 2018 through January 2021 in multiple respects. (Fugaro Decl., ¶ 32.)

Dr. Fugaro opines that when Dr. Nguyen-Traxler assumed care of Mr. Rideau in January 2016, she was

aware a GI specialist was following him but made no effort to understand her role in that relationship. She never inquired who the specialist was, what conditions were being managed, what testing was being ordered, or how often. She never requested or reviewed consultation notes from Dr. Ho, never spoke to her, and testified that she did not need to know what liver cancer screening her patient required because she was relying on Dr. Ho, despite having never communicated with her. (*Id.*, ¶ 12.) Dr. Fugaro indicates this failure was significant because Dr. Nguyen-Traxler was the only physician who could place orders for AFP testing and abdominal ultrasounds at Bayberry, and the results of those tests came back to her rather than to Dr. Ho. (*Id.*, ¶¶ 12, 13.)

Dr. Fugaro opines that Dr. Nguyen-Traxler had a responsibility to be aware of health care screening requirements and tests for her patients with extended stays, including mammograms, colon cancer screening with colonoscopies, and in patients with significant hepatitis C induced cirrhosis, awareness of the very high risk of hepatocellular carcinoma and the guidelines for screening such patients. Even if she was unaware of those guidelines in patients with severe cirrhosis, Dr. Ho's notes outlined the need for screening, and Dr. Nguyen-Traxler had a responsibility to read those notes and thus to be aware of the need to screen for HCC. Even if Mr. Rideau had stopped being evaluated by Dr. Ho for whatever reason, Dr. Nguyen-Traxler had a duty to continue the previously outlined regimen for HCC screening that she had ordered in the past.

Dr. Fugaro therefore disagrees with Dr. Hendifar that Bayberry and Dr. Nguyen-Traxler did not have an independent duty to order blood work or ultrasound studies for purposes of routine HCC screening. Dr. Fugaro also disagrees with Dr. Hendifar that Dr. Nguyen-Traxler did not have a duty to recognize her patient's lapse in care with Dr. Ho. Under the standard of care, she should have known and documented the schedule of consultations with Dr. Ho's office and directed Bayberry staff to facilitate the appointments. Dr. Fugaro states that the failure to do so resulted in a situation where Mr. Rideau's mother, who had no professional duty of care to schedule her son's appointments, forgot to reschedule, and with no schedule maintained by Dr. Nguyen-Traxler, Mr. Rideau was lost to Dr. Ho's follow up. (*Id.*, ¶ 35.)

Dr. Fugaro opines that the standard of care compelled Dr. Nguyen-Traxler to be much more attentive in response to Decedent's significant weight loss when she wrote a progress note on August 20, 2020, documenting that he had lost 42 pounds in the last six months. In a patient with known severe cirrhosis and an elevated risk for HCC, unexplained weight loss of that magnitude should prompt the possibility of liver cancer as a first consideration, and a reasonable physician should undertake a rapid and complete workup rather than attribute the loss to diet and pandemic isolation. Dr. Nguyen-Traxler had no current cancer screenings for her patient, and in the absence of a workup ruling out HCC, a reasonable and prudent primary care physician could not assume the weight loss was a positive development rather than a sign of malignancy. As Mr. Rideau continued to rapidly lose weight, Dr. Nguyen-Traxler continued to characterize the loss as desirable, which could have discouraged Bayberry staff from investigating further. She had a duty to make accurate chart notes, but even after the last AFP test in June 2018 she continued through November 2020 to note that AFP was normal and there was no evidence of HCC or hepatoma, without any testing to support those entries. (*Id.*, ¶¶ 20, 35.)

Dr. Arora opines that the 2.0 cm lesion identified on the September 2019 ultrasound was not a benign cyst as contended by Dr. Gaensler, but rather a suspicious lesion inconsistent with simple cyst criteria

due to its internal echoes and irregular, ill-defined margins. (Arora Decl., ¶ 21.) Dr. Arora further opines that the standard portal venous phase CT obtained in the emergency room on September 18, 2019 was an inadequate follow-up study because hypervascular lesions such as HCC are not visible on that type of imaging, and that MRI would have detected the lesion. (*Id.*, ¶¶ 22, 34.) Dr. Arora additionally opines that Decedent's liver function, assessed by Child-Pugh classification, would have made him a candidate for ablative therapy in 2019, directly contradicting Dr. Hendifar's opinion that no curative treatment was available. (*Id.*, ¶ 35.)

Dr. Arora further opines that Dr. Gaensler's identification of a 5.91 x 4.10 cm hilar tumor on the September 2019 CT is inconsistent with the subsequent imaging record. Dr. Arora documents that the area identified by Dr. Gaensler as tumor measured approximately 3.6 x 3.2 cm on the November 2020 CT and 3.4 x 3.2 cm on the January 2021 CT, a reduction of nearly 40 percent in the absence of any treatment. Dr. Arora notes that an untreated malignant tumor usually does not shrink, and this progression evidence is inconsistent with Dr. Gaensler's characterization of the finding as advanced HCC. (*Id.*, ¶ 33.)

Dr. Arora's states "I disagree with Dr. Hendifar that Mr. Rideau did not die of untreated HCC to a reasonable degree of medical certainty." As support for this, Dr. Arora identifies the very high AFP levels in December 2020 and January 2021, enhancing tumor mass in the portal vein, pleural effusion, lung nodules, and infiltrating lesions on CT, all in the context of cirrhosis. (*Id.*, ¶ 36.) In Dr. Arora's assessment, had the 2019 lesion been evaluated with MRI, AFP testing, or biopsy, Decedent could have been treated with therapy such as ablation. Instead, no additional evaluation was performed, and by October 2020 he had a 3 cm lesion and additional findings consistent with the progression of untreated early stage HCC over the intervening period. (*Id.*, ¶ 37.)

Dr. Graves opines that had AFP and abdominal ultrasound screening been performed every six months as required by the standard of care and Dr. Ho's plan, the 2.0 cm lesion identified on the September 2019 ultrasound would have prompted follow-up MRI imaging, leading to a diagnosis of early-stage HCC between September 2019 and April 2020, at a point when multiple treatment options were still available. (Graves Decl., ¶ 25.) Dr. Graves agrees with Dr. Arora that the 2.0 cm lesion was most likely a tumor and not a cyst, and states that in retrospect it was likely early stage HCC that went undiagnosed until January 2021. (*Id.*, ¶ 23.) Dr. Graves states that if Decedent had the advanced findings on the September 2019 CT described by Dr. Hendifar, Decedent would not have survived to January 2021 without treatment. (*Id.*, ¶ 24.)

Dr. Graves states that in 2019 multiple treatment options were available for early stage HCC that were never offered to Decedent. Dr. Graves says there were minimally invasive procedures that could achieve reasonable tumor control and extended survival, with recurrent small lesions amenable to re-treatment for several years, and which would have given Decedent better than a 50 percent chance of survival for five or more years. (*Id.*, ¶ 26.) Dr. Graves states that the five-year survival rate for patients whose tumors are detected at an early stage and who receive treatment is 60 to 70 percent, and because Decedent was "full code" he would have pursued treatment options were available to him. (*Id.*, ¶ 27.) Dr. Graves further opines that the delayed diagnosis caused Mr. Rideau significant pain and suffering. According to Dr. Graves, the medical records reflect that in the final month of his life Mr. Rideau was very ill, weak, and in pain from advanced cancer, yet received no pain medication

until he was placed on hospice because no cancer diagnosis had been made. (*Id.*, ¶ 28.)

Having reviewed the parties' evidence, the Court turns to Defendants' reply arguments.

Defendants argue on reply that Plaintiffs cannot prove causation beyond speculation because there is no evidence that Decedent had HCC, and Plaintiff's expert Dr. Arora stated only that the September 2019 lesion was "highly suspicious" for early HCC. (Reply Brf., p. 3:4-5, citing Arora Decl. ¶ 34.) This argument fails for three reasons.

First, Dr. Arora also stated, "I disagree with Dr. Hendifar that Mr. Rideau did not die of untreated HCC to a reasonable degree of medical certainty. In December 2020 and January 2021, he had 'very, very high levels' of alpha-fetoprotein (AFP), enhancing tumor mass in his portal vein, pleural effusion, lung nodules, and infiltrating lesions shown on CT. In the context of cirrhosis of the liver, the cause of his death is HCC." (Arora Decl., ¶ 36.)

Second, Defendants' position is undermined by their own expert testimony. Dr. Gaensler opined that HCC was the more probable diagnosis. (Gaensler Decl., ¶¶ 6, 8.5, 8.5(c), 8.5(f) 8.5(g).) Dr. Hendifar stated declaration filed with the moving papers that Decedent "certainly" died due to complications related to metastatic liver cancer, and that HCC was the reasonable presumptive diagnosis. (Hendifar Decl., ¶ 8.)

Third, if Defendants wanted to raise this as a potentially dispositive issue, they needed to raise it in the moving papers. Defendant's Fact No. 72, which is the only fact asserting lack of causation in the separate statement, asserts: "No negligent act or omission by [Dr.] Nguyen-Traxler...was within a reasonable degree of medical probability, the legal cause of Dwayne Rideau's death," citing to Dr. Hendifar's Declaration at paragraphs 8, 11, and 12.0 through 12.9, Dr. Gaensler's Declaration at paragraphs 8 and 8.1 through 8.6, and Dr. Rosen's Declaration at paragraph 13 as evidentiary support.

The Court has reviewed the evidence cited in support of Defendants' Fact No. 72 and finds that it is not sufficient to shift the burden on a theory that Plaintiff cannot prove causation because HCC cannot be confirmed as a diagnosis. Dr. Hendifar's causation opinions at paragraphs 12.0 through 12.9 all assume HCC as the diagnosis. Dr. Gaensler states that the September 2019 tumor was "likely hepatocellular carcinoma." Dr. Rosen's declaration and her conclusion that "no amount of follow up, scans, labs or treatment was going to change the unfortunate progression of his disease process" likewise assumes HCC. Defendants' experts do not say that AFP and ultrasound surveillance protocol would have failed to detect whatever tumor was present, or that causation in this case fails because HCC cannot be diagnosed definitively.

The reply asserts another new argument, which is that Plaintiff does not raise a triable issue fact because a patient has the right to control healthcare decisions, and Plaintiff did not show that Bayberry could have, within the standard of care, "overrule[d] the independent decision-making by Mr. Rideau...through his surrogate decision-maker, Marlene Rideau." (Reply at 3:23-4:1.) "Points raised for the first time in a reply brief will ordinarily not be considered, because such consideration would deprive the respondent of an opportunity to counter the argument." (*Jay v. Mahaffey, supra*, 218 Cal.App.4th at 1538 [cleaned up], quoting *American Drug Stores, Inc. v. Stroh* (1992) 10 Cal.App.4th 1446, 1453.)

The argument also fails on the merits as far as it goes. Defendants seize on Dr. Fugaro's use of the

term “surrogate decision-maker” to describe Marlene Rideau’s functional role as the family member Bayberry communicated with about her son’s care. Ms. Rideau cancelled one appointment and said she would reschedule. There is nothing to suggest that Ms. Rideau refused specialist care on her son’s behalf, or had the authority to do so.

The parties present competing experts on standard of care and causation. Plaintiff’s experts opine that Dr. Nguyen-Traxler failed to maintain the HCC surveillance established by Dr. Ho, and that earlier detection of the September 2019 lesion would have allowed treatment at a time when Decedent had a greater than 50 percent chance of survival. Defendants’ experts, on the other hand, opine that Dr. Nguyen-Traxler reasonably relied on Dr. Ho to manage cancer surveillance and no earlier diagnosis would have altered the outcome. These conflicting opinions create triable issues of material fact on the first and fourth causes of action.

The Facility Defendants do not argue that they are entitled to summary adjudication independent of Dr. Nguyen-Traxler’s liability. Because the Court finds triable issues of material fact as to whether Dr. Nguyen-Traxler complied with the standard of care and whether her conduct was a substantial factor in causing the harm, the Court need not separately address whether Bayberry staff met the standard of care.

Summary adjudication of the first and fourth causes of action is denied.

B. Second Cause of Action for Elder Abuse (Issue No. 2)

To prevail on an a cause of action for elder neglect under the EADCPA, a plaintiff must prove by clear and convincing evidence that the defendant: (1) had responsibility for meeting the basic needs of an elder or dependent adult; (2) knew the elder or dependent adult could not provide for those basic needs; and (3) denied or withheld goods or services necessary to meet those basic needs, either with knowledge that injury was substantially certain or with conscious disregard of the high probability of injury. (*Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 369, 406–407.)

Heightened remedies under the EADCPA do not apply to professional negligence; plaintiffs must prove something more than negligence—reckless, oppressive, fraudulent, or malicious conduct. (*Fenimore v. Regents of University of California* (2016) 245 Cal.App.4th 1339, 1348–1349.)

Where the claim is asserted against a corporation based on employee conduct, the plaintiff must show that an officer, director, or managing agent had advance knowledge of the employee’s unfitness and employed the person with conscious disregard of others’ rights or safety, or authorized or ratified the wrongful conduct. (Welf. & Inst. Code, § 15657(c); Civ. Code, § 3294.)

Defendants’ motion for summary adjudication of the elder abuse cause of action fails on two independent grounds.

First, on the question of recklessness, UMF No. 73 frames the argument as follows: “Because there was no breach of the standard of care to support a claim for medical negligence against Defendants, that necessarily means that no conduct by Defendants rose to the level of dependent adult abuse because dependent adult abuse requires much more egregious acts of malfeasance than the lesser claim of medical negligence.” Defendants make no independent argument that conduct falling below the standard of care would nonetheless fail to constitute recklessness. Because the Court finds that triable issues of material fact exist as to whether Defendants breached the standard of care, as

discussed in Section IV.A above, the elder abuse cause of action cannot be summarily adjudicated on this basis.

Dr. Fugaro opines that Dr. Nguyen-Traxler made unfounded assumptions, over an extended period, that Decedent was undergoing appropriate screening when he was not, that he was under the care of a liver specialist when he was not, and that his rapid weight loss was benign when it was in fact a manifestation of liver cancer. He further notes that she continued to enter progress notes stating "AFP normal, no evidence of HCC or hepatoma" despite the absence of any testing to support those conclusions. According to Dr. Fugaro, even after she began to suspect malignancy, Dr. Nguyen-Traxler failed to act with the urgency required in following up with the specialists, and she failed to inform Decedent's family of his significant weight loss or her concerns in time for them to advocate on his behalf. (Fugaro Decl., ¶ 36.) Viewed in the light most favorable to Plaintiff, this evidence is sufficient to raise a triable issue of fact as to reckless neglect. (See, e.g., *Sababin v. Superior Court* (2006) 144 Cal.App.4th 81, 89.)

On the corporate liability issue, Defendants argue that Plaintiffs lack evidence that any officer, director, or managing agent of GHC had knowledge of Decedent or authorized or ratified any wrongful conduct. (Mot., pp. 29:14–31:9; UMF No. 73.) Defendants have the burden-shifting backwards. A defendant moving for summary judgment may carry its initial burden either by presenting evidence that conclusively negates an element of the plaintiff's cause of action, or by presenting evidence that the plaintiff does not possess and cannot reasonably obtain the evidence needed to support that element. (*Aguilar v. Atlantic Richfield Co.*, *supra*, 25 Cal.4th at p. 855.)

Defendants did neither. The managing-agent issue does not appear anywhere in Defendants' Separate Statement of Material Facts, and the declarations cited in support of UMF Nos. 73–75 (those of Drs. Hendifar, Gaensler, and Rosen) address only the elements of a professional negligence claim. None of these declarants offers evidence directed to the requirements for corporate liability under the EADCPA. Because Defendants failed to meet their initial burden on this issue, the burden never shifted to Plaintiffs to demonstrate a triable issue of fact regarding corporate liability.

Summary adjudication of the second cause of action is therefore denied.

C. Third Cause of Action for Violation of Resident Rights (Issue No. 3)

The third cause of action is for violation of Health and Safety Code section 1430 which provides that "[a] current or former resident or patient of a skilled nursing facility. ... may bring a civil action against the licensee of a facility who violates any rights of the resident or patient as set forth in the Patients Bill of Rights in Section 72527 of Title 22 of the California Code of Regulations, or any other right provided for by federal or state law or regulation." (Health & Safety Code, § 1430(b).)

Defendants' motion for summary adjudication of the third cause of action rests entirely on the premise that because Dr. Nguyen-Traxler and Bayberry met the standard of care, there was no violation of Decedent's rights. (Mot., 32:28–33:4; Defs.' SSF No. 76.) In other words, Defendants treat the third cause of action as rising or falling with the negligence claims.

Because the Court has found triable issues of material fact, summary adjudication of the third cause of action must also be, and is, denied.

D. Punitive Damages (Issue No. 5.)

Defendants seek adjudication of punitive damages solely on the theory that the elder abuse cause of action fails, presenting no independent ground for adjudication of the prayer. (Mot., 33:1-5; Defs.' SSF No. 79.)

Because the Court denies summary adjudication of the elder abuse cause of action, summary adjudication of the punitive damages claim is likewise denied.

V. Disposition

Based on the foregoing, the motion is denied in its entirety.

14. 9:00 AM CASE NUMBER: MSC22-00100

CASE NAME: RIDEAU VS GHC OF CONTRA COSTA, ET AL.

HEARING ON SUMMARY MOTION

FILED BY: ANN NGUYEN-TRAXLER, M.D.

TENTATIVE RULING:

Defendant Ann Nguyen-Traxler, M.D.'s Motion for Summary Judgment, or Summary Adjudication of Issues, is **denied**.

Background

This motion is one of two summary judgment motions in this case heard on the same calendar. The Court's ruling on the companion motion filed by GHC of Contra Costa, LLC, Life Generations Healthcare, and Dr. Nguyen-Traxler in her capacity as Medical Director (the "GHC ruling") is incorporated herein by reference. The factual background, the summary judgment standard, and the substantive analysis set forth in that ruling apply here and are not repeated. The evidentiary rulings in the GHC ruling are likewise incorporated.

Procedural

Code of Civil Procedure section 437c, subdivision (f)(1) provides in pertinent part: "A motion for summary adjudication shall be granted only if it completely disposes of a cause of action, an affirmative defense, a claim for damages, or an issue of duty." (Nazir v. United Airlines, Inc. (2009) 178 Cal.App.4th 243, 251 ["Summary adjudication must completely dispose of the cause of action to which it is directed."] A motion that seeks to eliminate liability in one capacity while does not dispose of an entire cause of action and therefore does not satisfy this requirement.

Thus, the motion is procedurally improper to the extent it seeks summary adjudication of any cause of action based solely on a particular role. In addition, neither this motion nor the GHC motion differentiates between what Dr. Nguyen-Traxler did or did not do as attending physician versus Medical Director, or identifies any distinct duty arising from one role but not the other. Thus, even if the Court could summarily adjudicate a cause of action based on a role, the motions do not establish the entitlement to adjudication on such a basis.

Merits